

A young man and a woman sitting on a bench in the waiting area of the psychiatric emergency room, a crowded hallway in the old modular compound at the edge of the hospital grounds. The man is wearing jeans, has slightly long hair and the look of a university student. He is pale and tense, moves restlessly, and gazes through the room with a sense of imprisoned rage. The woman is older, wears a jilaba and a headscarf. The man was brought that morning by an ambulance of the Protection Civile, on a police order, RP, "requisition de police."

The head nurse keeps an eye on the patient, for until the psychiatrist on call decides on his hospitalization or his release, the patient is held by the police on garde à vue. They came from Tiflet, the head nurse tells me, a town that was once just a weekly market where tribal populations met, which became an army station during the colonial period and is today a growing urban sprawl in the rural hinterland of Rabat. The woman is the patient's mother—it is she who filed the police complaint. I try to imagine what exasperation, what lack of issue, led the mother to that resolution. I am waiting for Dr. R., a young woman psychiatrist, with whom I have agreed to share the day on call.

Some years later, this text is written in the margin of the encounter that took place that day, in light of encounters that followed with other patients and in different circumstances, but in the wording and concrete features of a particular life, as that life presented itself to me that morning in the emergency room. My reflections, in the form of a circumstantial commentary, address questions raised by the young man himself in his speech and related to his perception and imagination of "modern times" in its confrontation with a "cultural

home” he experiences as unlivable. It is a confrontation that takes place on the battlefield of his own body and life, but that by its nature concerns a whole society—as in a way a society, and the possibility of a common life, are called in by his speech. Spoken from the midst of his uncanny experience (which is partly inaccessible to his interlocutors), his questions point to the tear of a failing transmission—the inability to inhabit a tradition. They voice the violence of cultural identity in the postcolonial nation and an essential homelessness, which is that of his madness but is also, in a different sense, the experience of his generation. Yet, at the same time, his questions bear witness to a struggle for existence and creative elaboration, for a possibility of resymbolization, fought on the very ground of the ER interview. Punctuated by a reiterated request for a lawyer, “a lawyer who may speak in my behalf and defend my right,” his discourse is a paradoxical appeal to the law, the law of the state and the institution of psychiatry, in a plea to be recognized as a subject.

July 1999—it was my second summer of fieldwork at the Hospital Arrazi in Salé, Morocco, still somewhat the beginning. Since then I have been working with psychiatrists and patients, in the services, through the daily staff meetings and in the ER, in the attempt to grasp the experience of illness between different and sometimes incommensurable and antagonistic vocabularies of the self and to seize the specificity of utterance in that unstable intermediate space where speech is at once silenced and produced.¹ On one side is the psychiatric institution, with its diagnostic and therapeutic apparatuses, its styles of reasoning, its codes and its debates, the socialization of the residents into a mode of being, an academic hierarchy, and of the patients into their psychiatric illness. On the other side are the etiologies and therapeutic techniques associated with trance and the jinns in the practice of Qur’anic healers and in the experience of patients, often the same patients, outside the hospital. The question of culture was felt at the hospital as a central and a troubling one, pointing to a malaise difficult to locate, spoken in terms of the “cultural factor” in psychopathology, in the expression of symptoms and the themes of delusion, in the beliefs of patients, as a problematic attachment to a lingering and ineffective cultural tradition. It was the reason why I had been welcomed, an anthropologist, as an observer and an interlocutor. Yet despite this critical concern, approaches inspired by ethnopsychiatry or transcultural psychiatry were looked upon with diffidence, due to a feared folklorization of culture and to the colonial history that had constituted Moroccans into “cultural subjects,” as against the universal subject of the Métropole. But difference, the *différence* of oneself, resurfaced with uncanny reminders: as the enigma of a word, for instance, jotted down in French by the resident on call in the admission report as an approximate translation of something the patient said in Arabic or Berber

to describe an affect or a vision (French is the language of hospital files and all reports, as well as of the staff meetings and all medical school instruction); a word that returned as a puzzle during the staff discussion of the patient's case, materializing the chasm of institutional translation. What did the patient actually say in Arabic? What was the spiritual entity evoked? How do you express suicidal feelings, or speak of pleasure, in Arabic? A chasm of cultural-linguistic translation doubled the chasm of experience, bringing it as if to visibility—the experience of madness, and the inadequacy of all translation, within and beyond the perception of psychiatry.

From one fragment of life, or encounter, to another, during my time at the hospital I attempted to apprehend a scene marked by an urgent request (by the patients for listening, for recognition, for care) and, at the same time, by the impossibility of fully inhabiting the psychiatric institutional references as well as the cultural reference itself—the collectively sanctioned sites and vocabularies of healing. This is a central theme in the narratives of the women and men I met in the hospital and is a crucial site of reflection in my work. It exposes the conundrum of an attachment impossible to dissolve and of an ambivalence impossible to reduce to an unproblematic belonging; a conundrum that dispossesses the subject of the capacity to invoke the authority of the reference from which it draws its identity, all the while being seized in its matrix and its passion.

I understand this double impossibility as a complex effect of the repression of indigenous medical practices and of the understanding of illness and the body steeped in the unconscious archives of the subject under colonial and postcolonial rule. (I take “repression”—with Freud—in a literal/symbolic sense: violent political censorship, involuntary forgetting, psychic obliteration). The government of health played a crucial role in the establishment of the Moroccan colonial state; territory and subjects were conjured through sanitation, by way of vaccination campaigns and the work of mobile medical military units, while the exercise of “indigenous healing” was made punishable by law. At independence the postcolonial state inherited the colonial medical infrastructure and the legacy of the modernizing project, but failed to develop an effective social medicine. Despite the vision of late colonial and early postcolonial medical reformers, several among them psychiatrists, little efforts were made by the postcolonial state to provide health care for the growing number of impoverished and displaced populations. The result is today the *de facto* exclusion of large sectors of the Moroccan population from concrete access to health care—a realization that makes any claim to health rights essentially empty.

On this complex and ambiguous scene there is on the one hand the predicament of Moroccan psychiatry, with its colonial legacies as well as a history of early postcolonial experiments in deinstitutionalization and alternative care. It is

a history that accounts for certain indeterminacy in diagnostic criteria, allowing for flexibility and some inventiveness in the margins of institutional practice; a history that is specifically modernist, in that it registers the experience of a rift, a desire for an elsewhere—in the form of a hospital “bed.” On the other hand there is the continuing presence of other therapies and understandings of illness within a vernacular and a now increasingly revivalist theological framework, other practices, largely unrecognized and rendered incommensurable on the grounds of the hospital and in the larger discourse of national public health but that today foretell the possibility of different futures. The growth of Islamic healing in particular, in its multiple approaches and dimensions (spanning from the treatment of jinn possession in the path of God’s law [*‘ilāj al-sharā‘i*] to Islamic psychology [*‘ilm al-nafs al-islami*] and its debates with the natural sciences) registers a crucial shift in the existing coordinates, calling for a relegitimation of the grounds of belonging in terms of a renewed actuality of concepts, interpretations, and therapeutic interventions drawn from the ethical and theological traditions of Islam. So much is expressed in the hiatuses, the silences, of encounters in the psychiatric ER.

Before turning to the ER interview, some context is helpful to situate the institutional history, elucidate the terms of the questions, the actors at play, and the stakes.

Hospitalization by RP is not uncommon at this hospital, one of two main psychiatric hospitals and schools of psychiatry in Morocco, where what psychiatrists call *les pathologies lourdes*, “serious psychiatric disorders,” are routinely addressed. This is consistent with the 1959 royal decree regulating psychiatric treatment in Morocco, the law still in use today. Issued three years after the independence of Morocco, the decree (*dahir*) of Psychiatry is a modernist law of humanitarian inspiration, conceived with the explicit intent of protecting the rights of patients against the possibility of familial and political abuses. By this law all decisions concerning issues of mental health and the hospitalization of patients are put under the responsibility of psychiatrists. The law mandates the establishment of mental health centers in all provinces, limits hospitalization to three weeks, renewable up to a maximum of six months, enforces and regulates detailed clinical records in the hospital files, and states that hospitalization takes place “at the request of the patient, or of a public or private person acting in the patient’s interest, or in the interest of Public Order.” A family member or a public official may request hospitalization by filing a police complaint: but the final decision rests with the psychiatrist.²

In the wording of the law, specialized medical knowledge—the ability to discern “anomalies of behavior” and attribute a meaning to them—is at once a judicial tool and a guarantor of the patient’s freedom. Its modernist mission is attached to the responsibility of each individual physician, called to the ambivalent task of incarnating the law. As Gyan Prakash (1999) has suggested for the case of science in colonial India, psychiatry is constituted by this law under the ambivalent sign of surveillance and freedom.

Protecting the rights of patients in 1950s Morocco implied that patients be conceived in their individuality as subjects of care solely qualified by their (psychiatric) illness, understood as exclusively their own—not as the expiation of a past or collective history, not as the expression of an interpersonal or societal malaise, not as the effect of an external agency: a premise health practitioners themselves understood to be a fiction—and the site of a struggle. The 1959 Dahir of Psychiatry is a biopolitical document. Patients, *les malades* of this law, are subjects of human rights in as much as they can be represented as stripped of all attachments and relations, culture and belief, reduced to the “bare life” of their illness—outside of any prior inscription of living in a form of possible collective life, a symbolic order, with its intergeneration exchanges and its transference of memory and oblivion, which alone can provide a sense of ground and the possibility of choice, and creative improvisation (Arendt 1971; Agamben 1998).³

Yet in a context in which psychiatric care touches a small fragment of the population, and this not just because of a much debated cultural reluctance, but also due to the scarcity of institutional means (a total of some three thousand beds in specialized psychiatric hospitals located in the major cities, for a population of thirty-one million, and a dissemination of Centres de Santé in rural areas, understaffed and underequipped and operating in conditions of chronic emergency),⁴ the biopolitical project of the *dahir* takes on a specifically local dimension, one that is both utopian and exemplary. It institutes the site of an elsewhere by ambiguous contours, in rupture with the cultural institutions, where a recourse is possible in moments of despair; but also a force, which can seize one by the legitimate and arbitrary violence of the state, as in the case of the youths from poor urban neighborhoods brought to the ER by the police at night.

A patient’s file at a morning staff meeting. Overnight admission, a young man brought to the emergency room at 4:00 A.M. by the police. Had checked himself in at the police station of Taqaddoum at 3:00 A.M. Taqaddoum means “progress.” It is an underprivileged neighborhood at the outskirts of the old colonial center of Rabat. The youth declared to the police that he intended to commit suicide. He

was drunk. Was put under arrest, handcuffed, and taken to the psychiatric ER. To the psychiatrist on call he told a confused story that hinted at his rage and guilt toward his family, his father, of whom he refused to speak, his mother, remarried, and who in her helplessness to help her son paid for the wine with which he got drunk, and a girlfriend, the only person who understood him, but who had left to join her family who had migrated to Italy. He had studied until the fourth year in high school, then had dropped out, in a moment of rage for which he blamed his family. He spoke of the growing sense of failure over the previous months, his inability to find a job, his reality of chomeur.⁵ Buried in his room, "I have become a woman, they provide for my food." The resident who admitted him stressed his sense of impotence, emasculation, his ideas of destruction and ruin. Long discussion about the case, used by the senior psychiatrist of psychodynamic orientation to lecture the residents about the importance of listening for existential sense in the trajectory of a person's life. The steps taken by the patient were unusual, a powerful request for help, a request for authority, for the law. What was the sense of that recourse to the police? he asked. The police as a symbol of power, of violence, castration. The patient made a recourse to the law as the site of symbolic mediation: but the law handcuffed him literally, and he felt let down, was furious. The patient's spectacular search for help suggested that the problematic of the subject was not psychotic, the psychiatrist said (that is, there was an attempt to express something symbolically through an act or a symptom, here perhaps to send a message to his family), inciting the resident who took the case to actively listen for sense. The next day the young man was discharged. The resident who had taken the case (reluctantly, because such patients are difficult, they need time, affect us, and there is no time) said that upon awakening the patient was sober and denied the truth of anything he had said the night before, attributing it all to the effect of alcohol. I met the young man with the psychiatrist, his words were impenetrable; I felt like asking more, but didn't. He was coherent and reasonable, and the young psychiatrist signed his discharge, said that there was no reason to hold him at the hospital, it would be an abuse. There was a thick pile of patients' files on the table, we were in a small lounge inside the men's locked ward, patients waiting outside the door, walking in the door, asking for attention, much more visibly ill than that young man.

Reflecting on questions of public health in the *banlieues* of French cities and in the context of irregular migration, Didier Fassin (2001, 1999, 2000) argues that the biopolitical reduction of being to "bare life," to the silence of a suffering body, grants to the excluded an ultimate and paradoxical recourse to the law. He names this agency "biolegitimization," a right issued from the loss of all rights and communal affiliations, and shows how the biopolitical operation is literally incarnated by migrant subjects of irregular status, for whom illness becomes the only possible ground of appeal in their request for inclusion. In Fassin's view biolegitimization is not a tactic of resistance, but a strategy of power on the shifting terrain of European late capitalist and postnational societies. Its rules are dictated by the ideological and juridical reconfiguration of the republican state, which reproduces inequality and racial exclusion as it pro-

motes the values of *égalité* and which has come to confer an absolute moral primacy to the nonproductive life of the suffering foreign body. In a context in which eligibility to basic rights is argued and granted on the basis of an incurable illnesses (such as AIDS or lead poisoning), one must question the ideological-legal structure of a democratic state to which migrants gauge their bodies in a mortal exchange. But furthermore and unlike the case of Europe, in the situation I attempt to apprehend, becoming a biopolitical suffering body does not grant a paradoxical juridical inclusion. The appeal to the institution of care must be understood differently, in a space of impossibility that is also the ground for the imagination of life. This chapter traces a comparably paradoxical gesture of appeal, an appeal to the law of the state, the “constitution of the king,” but an appeal that does not provide an access to citizenship because it rejects existing ties and affiliations and is instead spoken in an ambiguous utopian zone, where creation and loss are intertwined in the singularity of a marginal life.⁶

Overshadowing the phrasing of the 1959 *dahir* is a century-long colonial narrative characterizing the indigenous culture and family structures as obstacles to the development of individual responsibility and European-style citizenship. Leaving little room for uncertainty, discontinuities, experiential discrepancies, and for the recognition of historically different ways of being a subject, this narrative stresses the fundamental role of the patriarchal family in Islam, the overarching weight of the group, the force of a collective imaginary that makes it impossible for the individual to exist *qua* individual, and the weight of a religious law where the sacred is the voice of the group (see, for instance, Berthelier 1994; Igert 1955; and Pandolfo 2000). Superimposed on that narrative, however, and orienting the spirit of the 1959 law in an opposite direction, is an institutional reflection on the question of anomie in the context of colonization and modernization: a preoccupation with the crisis of traditional structures, and the unavailability of alternatives, in terms of which the new sciences of psychiatry and social psychology were situating themselves in Morocco in the 1940s and '50s.

In the writings of J. L. Roland (1957) a French psychoanalyst who became the first director of the Service Central de Psychiatrie et d'Hygiène Mentale at the Moroccan Ministry of Health at independence, as well as the first director of the newly created Hospital Arrazi in Salé, the focus is on processes of anomie in colonial modernity. The causes of mental illness are sought in the destructuring effects on the Ego of traumatic socioeconomic transformations: the rural exodus, rapid urbanization and overpopulation; the compression and promiscuity of living space in poor urban neighborhoods, where intimacy is systematically violated, producing in subjects a sentiment of intrusion bordering on psychosis; the crisis of paternal authority; the “dissociation of the family”; the

growth of aggression and self-destructive behavior caused by the unconscious guilt of having violated ancestral rules; the stress of bilingualism with the colonial language; the formation of an urban lumpen proletariat; the spread of alcohol and crime. The biopolitical production of “patients” is presented in this reading as the result of a historical process of uprooting, the effect of catastrophic social change, comparable to the production of refugees in a war. The task of psychiatry becomes that of understanding and healing these ruptures, without looking back to the past, in the imagination of a new society.

In the spirit of its modernist inception, the hospital has grown as a site of treatment, education, and research, taking an active role in the public debate on pressing social issues; but it is also as a space where the postcolonial *parti pris* of a technoscientific modernity precludes the recognition of vocabularies of illness other than those of psychiatry, and to a certain extent psychoanalysis. This is a political choice consistent with the modernizing strategies of Maghribi postcolonial states, which are today full of consequences and not free of disquiet.

In *La psychanalyse à l'épreuve de l'Islam* (2002), psychoanalyst Fethi Benslama interrogates the psychic consequences, for Maghribi and Middle Eastern subjects, of the destruction of traditional forms of belonging in the successive violence of colonization, modernization, and nationalism, with particular reference to the contemporary reinvestment of Islamic religious identities. He diagnoses a process of subjective revocation (*révocation subjective*), a collective regression to archaic forms of psychic life, which he relates to the gravity of the present social crisis and to what, in his view, is its most prominent psychopathological symptom: the quest for narcissistic reparation under the sign of what he calls a “return to the Origin.” In Benslama’s reading, which extends a Lacanian understanding of psychosis to collective phenomena, this process results in the disclosure of a chasm, a terrifying void, which is “filled,” he argues, with the psychotic fantasy of a return to an original identity—a return in which the subject dissolves. Contemporary religious movements, and the Islamic revival in particular, would be an instantiation of that kind of delusion. This chapter engages critically, if indirectly, with the questions raised in Benslama’s text and attempts to displace them, at once ethnographically and theoretically.

How to think the aftermath of the subject of psychoanalysis without necessarily resorting to the categories of psychopathology? How to address the present sociocultural, political, and spiritual sea change without surrendering to comforting psychosocial diagnoses? How to cultivate the disposition of a subject open to the risk and enigma of alterity, or in T. Asad’s words, “of subjects who can open their minds to something that is strange, or uncomfortable, or distasteful” (Asad 2002)? Some directions are found in the Freudian approach

itself. Identification, underpinning the processes of modern mass society as well as the formation of the Ego, is “more ancient” than the covenant in speech; it is a “passage through the Other” in which the “I” is both founded and revoked. In the instability of that “passage” is located at once the risk of radical loss and the ethical possibility of coexistence (see Lacoue-Labarthe and Nancy 1997; Borch-Jacobsen 1993). Developing this line of thought would lead, among other things, to questioning the irreducibility of the boundary between subjectivity and psychosis, exploring the troubling and unstable interfaces of subjectivity and depersonalization, as was pursued in European phenomenological psychoanalysis and is recently attempted, on the basis of different premises, in American cultural psychiatry.⁷

In relation to these questions, the story of Reda,⁸ the young man I was to meet in the emergency room, provides a number of unexpected answers. As with many youths at the onset of psychosis, or simply someone whose “skin” is less impermeable, Reda takes it upon himself to articulate a malaise larger than his own, that of a collectivity of which he becomes an echo. From the rage of the torture he feels, inflicted on him by a cultural home he experiences as unlivable, Reda moves to a creative interpretation of his madness, opposing to that intrusion, to that shrinkage of vital space, something other than a search for identity: a “creation” that transforms alienation from within the space of madness itself. His answer displaces the terms of the question and suggests that if we pay attention to the concrete specificity of particular lives, and the philosophical possibility of other forms of life, we may glimpse the inception of a renewed *kulturarbeit*, a novel articulation of unspeakable cultural truths—here, in the margins of a psychiatric encounter.

SHADOW THEATER OF MODERNITY

As is often the case in the emergency room, there is tension in the room. Dr. R. is opening the official seals, reading the letter from the police. Then she looks at the man and asks his name. The mother replies that his name is Reda and that he’s thirty. But Reda is not listening. He complains loudly about her, his mother, speaks French and addresses Dr. R. directly. Says that his mother doesn’t understand him, couldn’t possibly understand him, she is illiterate and ignorant, unable to even count properly.

Dr. R. interrupts him, in Arabic (visibly not to exclude the mother from the conversation, she doesn’t understand French): “U nta ‘arfti? [And you, can you do it?],” she asks. Reda is reluctant, annoyed by the obvious, finally says “forty-two” and insists that he shouldn’t be in this hospital, that he requests a lawyer, he demands his rights: “*Je demande un avocat, je demande mes droits!*”

"Elle veut me rendre fou [she wants to make me crazy], she forced me to consult with a *fqih* [Qur'anic healer] against my will." Reda insists on speaking French, ignoring the injunction of Arabic, the psychiatrist's refusal to reply in the language of his choice: the colonial language, language of the hospital files, but also, for him, language of his scientific studies and of his literary journeys, language his mother doesn't understand.⁹ It is the only language in which he is able to articulate his request, his appeal to the Other, embodied in the room by this frail young woman sitting behind a desk.

"Why should I take medications?" "*Takhod dwa!* [Take your medications!]," (alluding to his mother), "I should instead take a lawyer because of the suffering she is inflicting to me. As for this language I am speaking [French] she [his mother] is illiterate and we are entering the twenty-first century." He repeats this in Arabic, perhaps to make his mother feel the humiliation: "*Makatfehemsh lfranse, maqariash.* She is illiterate, doesn't understand French, while I have a *Troisième Cycle in Mathematics.*" (The mother signals to the psychiatrist that this corresponds to reality.)

I ask Reda about his education. "I have a university degree, a *maitrise* with equivalence in the French system, pure math, logic. I am a logician, only I disguise myself, and play instead my literary games [*je fais des jeux de littérature*]. I studied pure mathematics, pure logic, I have a degree in linear algebra, *'arfti ashna hia la géometrie algébrique?* [Do you know what linear algebra is?]" (violently to his mother).

"*Ma'arfa walu*, I don't know anything," submits the mother.

Reda turns to his mother aggressively and says in Arabic, "*'alesh dditini 'and llfqih* [why did you take me to the healer], against my will, and with the Fire Department? [The ambulance of the Protection Civile took him to the hospital that morning, but in his accusation the institutions overlap.] Why? To feed me herbs [*'ashub*] and potions, nutmeg [*lgusa*] and fennel [*habt lhalawa*], to subjugate my will. I can still understand a girl who does that, she desires you and puts *twkkel* in your food, feeds you a couscous with magic to make you marry her. . . . But this woman?"

And then to the psychiatrist, and to me, in a mixture of Arabic and French: "They made me swallow meals of God only knows what, dog and cow brains . . . garbage [*les déchets*], cow dung they mix in with couscous, and much much more. In our region laboratory experiments are made with couscous!"

Insisting on using Arabic as the language of the interview, Dr. R. asks Reda whether he really thinks that his mother put magic in his food. Reda replies that he is suspicious, he doesn't trust her, her cooking, her food. Exasperated, the mother denies having put anything in the food, and Reda replies that in any case her cooking is poisoning him, and he makes once again an appeal to the

mediation of the law, a law that might speak in his behalf—literally, to a lawyer: “I request a lawyer, for a lawyer who may defend my case, for all the suffering that I endured with this woman . . . [repeats in Arabic], because I have suffered so much . . .”

The scene in the emergency room becomes a trial in which Reda is at once plaintiff and defendant. It is a trial and a self-defense made possible by the summoning of the institutional code, calling subjects in the position of patients, protected by the recognition of their illness. Reda’s passionate self-defense, and his request for mediation, are a testimony of his struggle for creating a “place,” a space in which existence might begin. The absence of place, of horizon—in a physical and symbolic sense—is a poignant feature of Reda’s claustrophobic speech, aimed at pushing back an aggression, distancing an invader, striving to protect the possibility of a vital movement threatened at his core.¹⁰

Reda is upset, the atmosphere is tense. I recall the anxiety and the way his voice affected me; I remember thinking that the psychiatrist must be affected as well—because the drama that was unfolding in his voice, and before our eyes, resonated as something larger than a personal or a family affair. It overflowed the specificity of his personal history and resonated for me (and how could it not for Dr. R.) as a historical and political affair. In that overflowing, the possibility of a personal history was reduced (or imprisoned, in the vocabulary of *daseinanalyse*) in what L. Binswanger (1958) calls the splitting off of experiential consistency into alternatives, into a rigid either-or. Reda, his voice, his body, became a rift where intersected and clashed the cleavages of Moroccan modernity, the ideology of modernity, the same cleavages by which his life was furrowed and that underpin the possibility of this hospital scene. Speaking in the name of that which was attacking him, the problematic of the subject was to push back the offensive, in the two opposed camps of magic-tradition and psychiatry-modernity. The psychiatrist, the mother, and I were at once spectators and characters on the scene of this recital. Reda was in it by his struggle, his search for an exit, most of all by way of his rage.

TWKKEL: Topography of the Unlivable

As I listen back to Reda’s words in the transcript of that conversation, I attempt to follow the figures of his despair, analyze its images, as one follows the images of a dream.

He outlined the main themes at the outset, in a sort of poetic condensation. Poisoning and incorporation by ingestion, cannibalization, illiteracy, the theme of the Mother. She wants to make me crazy, she wants to kill me. And, “we are entering the twenty-first century”: exit and forward movement—science, mathematics,

logic, and geometry, education by the intercession of the foreign tongue. Murderous persecution by “her cooking,” her food and her magic, and by the intimate relationship his mother entertains with the *fqihs*, traditional healers and agents of traditional culture. The magic Reda refers to and specifically names is itself called *twkkel*, from the Arabic root *akal*, “to eat” (in this form of the verb, to be devoured, eaten away, destroyed). *Twkkel* is a technical term, the name of a magical operation and of a form of illness attack described in manuals of healing and magic, and it is a key notion in the vocabulary of affliction, in the discourse of healers as well as patients and their families. Unlike other conditions, more directly related to the jinns, complaints of *twkkel* are rarely censored in the illness narratives of patients in the psychiatric hospital, perhaps because of its perceived equivalence with psychiatric symptoms of persecution (by poisoning or, more culturally specific, in terms of a “delusion of sorcery”), or because of its more bodily, physical connotation. Intentionally harmful *twkkel* can be obtained by natural means (attempting to kill a person with poisonous plants or other toxic substances mixed in food or drink), but in its most dreaded form it is *sihar*, “witchcraft,” a quest for power involving the intervention of the jinns in the realm of death and the manipulation of the forces of harm.

In the explication of a healer with whom I discussed this point, if *twkkel*’s destructive force, as *sihar*, is channeled through magical operations that summon the malevolent intervention of a jinn, its origin, its source, is always in an envious passion, a devouring desire of the person to incorporate and/or annihilate the other. A desire and a rivalry, he added, that have become endemic in the current conditions of social injustice in Morocco and in the world, generalizing the model of *sihar* to a global struggle for power and destruction. (As we talked in the living room of his house, one of the rare concrete constructions alongside a shantytown in the outskirts of Rabat, this healer, who is also a preacher of Muslim revivalist orientation added, “Human beings exist in justice [*adami wajudi al-ʿadl*]. And justice is violated by coercion [*ikrah*]. There is no justice in this world. . . . There are laws, but there are people who are *too* powerful, and interfere with the law. How do the powerful prevent the exercise of the law? By pecuniary corruption (*maddian*), or by coercion and intimidation. In this situation the poor and powerless person turns to sorcery, because he or she has been offended and hurt. . . .”)

Unlike the healer I am citing, a real interlocutor and acute clinician and exegete, *fqihs* keep coming back as phantasmatic figures in the emergency room discussion with Reda and his mother. In Reda’s account they are persecutory agents, pointing to a secret complicity between his mother, his cultural milieu, and the forces of tradition in the insatiable passion to subjugate others, him, and *enslave his free will*. In his mother’s account instead, the mention of *fqihs* traces the

secret map of a quest for healing in the home town, the neighboring region, and all the way to Rabat; a quest the mother cannot fully acknowledge in the psychiatric context, where that idiom of illness is invalid (and in a way couldn't fully acknowledge in general, for the realm of the jinns is ontologically elusive), and that she imputes to Reda himself. It is he, she says, who speaks of the jinns, who complains of being possessed, who seeks the cures of a *fqih*, although admitting later to have herself sought their intervention repeatedly, even without her son's approval or physical presence, as when she stood in line with her son's photograph in her hands at 5:00 A.M. at a healer's door—for the popularity of the young *fqih* was such that petitioners had to take a number to be received, as one did in the hospital. That healer told her that Reda was *mqiws*, “touched” by the jinns (*tqas*, “had been touched,” registering the event of an initial demonic entanglement), and gave her an amulet she stitched in his pillow; yet another *fqih* diagnosed that he was struck by a *jinniya*, a she-demon, and prescribed the sacrifice of a ram. With great difficulty, she said, she gathered the money to buy the ram, and called the 'Issawa brotherhood to perform the trance dance, and brought in Reda with a stratagem (*b-l-hila*) so that he might be smeared with sacrificial blood, fall into a trance, and be healed. Reda danced all night to the rhythm of the music, but did not “fall” (*ma tah-sh*, that is, into a trance). In the end he got up and left and didn't come home for several days.

In Reda's account, the image of couscous is the condensation of his haunting, pregnant with what Freud called *unheimlichkeit*—the uncanny unfamiliarity of what is at once intimate and foreign. Drawn from the Maghribi repertory of magic, feminine magic in particular, couscous becomes for Reda a figure of the murderous forces of the Home that threaten his psychic and physical integrity. There are echoes in his words of well-known magical recipes aimed at destroying an enemy or subjugating another person's will, recipes mentioned in magical books, but even more in the anonymity and terrors of rumor: such as the secret practice of digging out a corpse in the cemetery and mixing the couscous with the hand of the dead, or seasoning it with excrement and other bodily scraps, before feeding it to the victim. Couscous is an emblem of the most familiar and intimate, the definition of the communal meal itself, prepared by the maternal hand on Friday and other holidays, offered as *sadaqa* (charity) to the poor, eaten in common at holy festivals in the space surrounding a sanctuary. In some legends, human settlements are said to have been founded around a *gsa'*, a large wooden couscous plate, as in a mythical narrative from a region in the Moroccan south about a *gsa'* full of couscous that descended from the sky brought by the angels, in the midst of an open space in the palm groves; people gathered to eat the miraculous meal, and from that gathering a community was born.

But for Reda the communal meal becomes a mortal threat. This possibility was already inscribed within the affective ambivalence of the image of cous-cous, metonym of the identification with the group, which disclosed its destructive pole in the magical use. Yet the historical contingency of colonial and postcolonial expropriations in Reda's experience, and the way that contingency took a specific form in his life and rehearsed itself through him, painfully and madly, suggests that we should approach that image as something more than cultural psychology or as the inherent reversibility of symbols. To belong to the "we" of the cultural identification means to swallow that meal and to be killed.

Later in the course of the interview Reda came back to this theme. Dr. R. asked the mother whether she had in fact taken Reda to a healer, or were his accusations unfounded? And the mother replied that Reda went to the *fqih* on his own, because he felt he was "inhabited" (*kan keygul fih jnun, rah ni mskun*). Reda interrupted her angrily, beginning in Arabic but quickly turning to French (because the real interlocutor was Dr. R., to whom his appeal was addressed): "Nti lly dditina 'and lfuqqaha f tiffet! [It is you, you took me (us) to the healers in Tiffet!] I never agreed to go. *Je suis maladif* [I am vulnerable and sickly], and you made me do unbelievable things . . . the *fqih* would order me to smoke his magical writing on the blue paper of the sugar loafs, to vomit and smoke my vomit, I don't know, this woman is going to kill me with her healers!"

"It was to cure you," says the mother.

In Arabic: "Cure me, she says! What cure, and what doctors? Why did you take me to the healer, to push me to my ruin? In French: Because I have eaten something, she fed me something, she'd grab me, she'd say eat, eat, eat! She said, go to the psychiatrists of our town, go to the healers!"

If eating and being fed/devoured imply the omnipotence of the Mother-Tradition, the illiteracy of the mother suggests impotence and amputation. It is an impotence that bears a sign of forbidden entrance, interrupted trajectory, lack of exit. Ignorance, paralysis, subordination to "tradition," blocked access to modernity, to language and critical thought: "*elle est analphabète et on rentre dans le vingt et unième siècle* [she's illiterate and we are entering the twenty-first century]."

The mother's illiteracy is the son's loss of voice, if he doesn't manage to get away, to distance himself, escaping their capture ("it was to elude the grip of these two *analphabètes*," he says later on in the interview, referring to his parents). The *we* of "we are entering the twenty-first century" indexes a desire of community, to rejoin those who "enter" the future, those who *have access*: to modernity, to language, to the self. Exit and forward movement: science, mathematics, logic, and geometry, education by the intercession of the foreign tongue. In this configuration "reading" understood as a form of resistance and

an inalienable right holds a central place in what should be understood as Reda's *requisitoire*, at once a self-defense and an indictment.

In "the twenty-first century" Reda finds sanctuary against the persecution of the "home," which returns to attack him in the Real, from the outside, menacing and deadly—a threat of amputation by the hand and the speech of his mother. His reiterated request for a lawyer is an appeal to mediation—literally, insulation; the insulation of a "skin" that may draw his boundaries, the mediation of the law of the state by which he may declare his right to exit, and of the law of discourse, the possibility of an authorized speech. In the economy of his saying, the use of French is also this.

At stake is Reda's struggle against the annihilation of himself.¹¹ The theme of persecution and of his "suffering" (*cette souffrance*) in the hell of his experience are the negative and tragic dimension of this: imprisonment and paralysis, cannibalization, estrangement of the self and affirmation of a right to existence by refusal of identifications—with his mother, his language, his tradition. The theme of Exit—through education, science and reading, and later through literature and the Imagination—raises the question of freedom.

"KANHARAB B-LA LITTÉRATURE [I escape,
save myself with literature]"

Dr. R. asks Reda whether he hears voices, "*Makatsma'sh aswat?* [Don't you hear voices?]," suggesting in her question that he might. It is a key question to establish the presence of psychosis and to possibly move toward a diagnosis of acute psychosis, paranoia, or schizophrenia. "Voices," auditory hallucinations, signal in psychiatric semiology the loss of connection with reality and of relation to other fellow beings that are characteristic of psychotic delusion. They are intrusive injunctions coming from the outside; they can be aggressive, injurious, give orders, or can provide an external commentary. In psychoanalytic terms, they witness a tear in the subject, a hole, filled by the work of delusion, that ultimate retreat.

The psychiatrist's question, asked routinely in clinical evaluations, marks a critical turn in the conversation, a metamorphosis in the scene of Reda's speech.

Makatsma'sh aswat? (Don't you hear voices?) is an ambiguous question in its Arabic formulation, albeit routinely asked in clinical interviews. Reda does not deny, he does indeed hear voices, attacks coming from the outside, just as this voice that is addressing him now. He assumes the place assigned to him by the institution and from there operates a displacement, wards off the threatening voice.

"At night," he replies to the psychiatrist, hinting that the invasive voices come at night, "*et alors je fais jouer les voix de Cervantes, comme si je faisais la comédie.*"

Kanharab b la littérature [and when that happens I rehearse Cervantes' voices, as if I was staging a play. I fight back with literature, I escape, and save myself with it—*kanharab b la littérature*].”

Then to his mother, in Arabic: “You don’t know Cervantes, you don’t know anything, this is suffering, you only know how to make couscous . . .

It is at that point that the anguish of persecution and imprisonment gives way to an imaginary experience of creation, a “flight” of the imagination, where Reda stages his characters in the open spaces of a literature in the foreign language.

We—his interlocutors—won’t witness the immediacy of his flight. Reda presents his experience in the form of a reflection, offered in the distancing mode of commentary. Because for him what is at stake is a “retreat,” an exit into another scene where the Self may be rescued and healed. Granting to it the status of a concept, Reda calls that other scene *le Stade Cervantes* (the Cervantes Stage). It is a “stage that exists in literature,” he explains, but that sometimes he experiences in life. This “Stage” (a term he uses in the double sense of stage in the theater and human developmental stage) has the configuration of an intermediate world. I say this with a term of comparison in mind, one to which Reda would not have referred explicitly: *Al-Alam al-Mithal* (the World of the Imagination) in the thought of the thirteenth-century mystic-scholar Ibn al-‘Arabi.

The literary identification by which Reda says to “escape” and “fight back” (I will return to the double play of his expression) oscillates between the roles of Cervantes-the-author and Don Quixote-the-character—between a controlled staging of madness in a Baroque self-reflexive literature and the madness without return of the wandering knight, prisoner of his imaginary combats, suspended between the books and the life. Yet, for Reda, all this happens in solitude, because rare are those who, in his surroundings, in his small town as in the emergency room, are familiar with Cervantes, Don Quixote, and the story of the windmills. And this independent of the fact that Cervantes, the historical author, was in his time influenced by the reading of Arab texts, and in particular by the narrative strategies of the *Arabian Nights* as well as, perhaps, by the reflection on madness and healing the *Nights* represent.¹²

But first of all the voices.

In French to Dr. R. and increasingly to me, Reda says: “I wake up very early in the morning just to read, because with the crisis of the economy, there are no jobs, there is so much unemployment, and we live in Yacob El Mansour [an underprivileged neighborhood in Rabat], there is so much noise during the day, it is impossible to read!” (Reda had registered my reaction at his mention of Cervantes, and a connection was created between us, a connection through reading. Physically foreign, with a foreign accent in both Arabic and French, a

“researcher,” from his perspective analogous to him, I am perceived with complicity and included as a character in his tribunal.)

In this as other public hospitals, where the majority of the patients speak vernacular Arabic and are unfamiliar with medical and psychiatric concepts, the staff makes use of an intermediate vocabulary supposed rhetorically to be “that of the patient” (*la langue du patient*). It is a language of trade, in fact, loosely codified and deployed in the effort to make possible clinical interviews across experientially incommensurable conceptual worlds. It is a vocabulary based on the implicit assumption of the one-to-one translatability of symptoms or, more precisely, of the referential transparency of psychiatric symptomatology, which, whatever its symbolic expression, will necessarily translate into its true referent, that of a recognizable disease entity. *Lwusuwas*, for instance, that demonic haunting, maddening whispering in one’s ear, temptation of unreason, which is a central figure of madness in the Qur’an as well as in everyday speech, becomes the equivalent of obsessive neurosis or, in the terminology of the fourth edition of the *Diagnostic and Statistical Manual of Mental Disorders* (DSM-IV), obsessive compulsive disorder. Hence to make a differential diagnosis the psychiatrist will attempt to isolate *lwusuwas* from the “voices” of schizophrenia.

Or when the presence of a jinn is at issue (in the complaints of the patient, never in the questions of the psychiatrists, unwittingly registering, by that reticence to enter the “patient’s language,” the vulnerability and risk of the “psychiatric operation,”¹³ the task is to detect the symptom behind: the question becomes whether the entity, or voice, is internal or external to the subject and what the patient replies to that voice. As a senior psychiatrist put it at a staff meeting, discussing the case of a young man possessed by a jinn who, rather than speaking through him, never stopped talking to him: “From a purely psychiatric point of view what we want to isolate is the type of symptom, beyond the anecdotal figure of the jinn, which is drawn from our available cultural repertory and is not interesting in itself. We know that there is something embodied [*incorporé*], which causes the patient to do certain things, which gives orders to the person. It could just as well be a device implanted by the CIA in his body. From the semiological point of view in all these cases we have one and the same symptom, which we recognize as *automatisme mentale*.” Such is the risk, and the violence, of concepts in translation, when translation no longer discloses the foreign nature of language, but becomes instead the apparatus of appropriation and reduction to the same.

Wash katsma’ aswat f-udnik? (Do you hear voices in your ear?) Formulating the question in Arabic is not obvious, even though it is done all the time. Because in the larger sense indexed by the Arabic language and rooted in the specific way of experiencing subjectivity exemplified in the discourse of the jinns, as

well as, at a more philosophical level, in the ecstatic dimension of religious experience, hearing voices is not equal to madness. Or at least a certain madness, a certain “exit” from the self, is foundational for the possibility of subjectivity and truth. The Qur’an was revealed in a visionary dream by the voice of an angel speaking in Muhammad’s ear. Revelation, as argued by Fethi Benslama (1988) in his book on the Qur’an, is the result of a moment of radical ultimate existential risk, a passage through madness in which the Prophet might be lost, but that instead gives way to the interpretation of the voice as divine utterance—an interpretation made possible, and literally engendered, by the feminine intercession of Mohammed’s wife, Khadija, on whose “lap” the divine’s message becomes audible and can be received as a divine utterance.

Later in the interview the mother tells of an episode that illustrates the function of “exit” Reda imputes to reading, to the use of a foreign language, and to that experience of journeying he calls literature. One day, toward the beginning of his illness, he took off with his books in the wilderness: “Once, when he was first sick, during the time when he was praying a lot, he vanished for three weeks, went out into nature [*kharj ll tabi’a*], into the countryside [*f-l-khala*], into the wilderness of open empty spaces. He spent there his days and his nights, he took his books with him, said he was going to read his book to the land! Imagine, three weeks like that, he walked from Tiflet to El-Hajeb through the countryside, he slept outdoors and begged for food.”

Reda opposes that all this is none of his mother’s business. He turns to Dr. R, and to me, in French: “I searched and searched in the books, I was forced to hide the Italian language, because you realize, *chez nous*, in Tiflet, it’s impossible to show it in public . . . because I love languages, I love reading, I love nature, it is my right, I request a lawyer immediately, it is my right to go out into nature and do what I wish, here in Morocco, it is my right! I respect the constitution of the king, and I demand a lawyer, a lawyer here and now!”

Kanharrah b la littérature: I save myself with literature. *Kanharrah*, a fleeing from an aggression, a dangerous situation, an attack or an invasion. A fleeing that is also a distancing, a departure that can save oneself. *Kanharrah*, I escape, resembles and in Reda’s use rhymes with *kanharrah*, “I fight back, engage in a struggle,” as in an armed counterattack, “I fight back with literature.” In this sense, for Reda literature is a “counterdelirium” comparable to the counterdelirium of witchcraft, according to Favret-Saada (1977), as well as to the counterdelirium of literature for Deleuze (1998). But unlike the aggressive metaphor of witchcraft, the saving doesn’t reestablish the Self as a presence, for what departs, in his image, what is no longer there, is precisely the self that is saved, that is both lost and saved. “I flee/fight back with literature,” that is by way of literature, but also literally “with it,” with the books on my back, for a life of wandering with “my”

characters, the other voices, on the stage of “my” comedy, Cervantes’ Stage, the mise-en-scène of the voices. Myself, Don Quixote, Cervantes’ mad reader and wandering knight. The Cervantes Stage is “another scene” where all persecuting others have vanished, including himself as a narcissistic injury. It is an “open empty space,” *l-khala*, where he can be alone and find sanctuary with his stories and his books. In this place Reda reads to nature: an empty listening, the only interlocutor that, he feels, doesn’t reduce him to silence.

Later, after his hospitalization, in a room in the men’s locked ward, I ask Reda about his encounter with literature. He tells me about the public library in his hometown, where people don’t read, are illiterate, and don’t speak other languages (they do speak Berber and Arabic!); he tells me about reading Voltaire and about his passion for Dostoevsky and Cervantes. He read *Crime and Punishment*, but it is *Don Quixote* that really had an impact on him. When I ask him why “literature,” he explains that since he started having difficulties in concentrating (this is how he speaks of the “strangeness” of his condition), he had to abandon his scientific studies and began to read—to dig in literature (*fouiller dans la littérature*). And why Cervantes? I asked. He returned the question to me, do I know the story of Don Quixote? “Don Quixote has read all the books,” he explains, “the books of world history, and he wanted to fight with the wind-mills, and for his love Dulcinea wanted to show to be a great knight, and this is why he took on traveling with his friend Sancho, but most often, in fact, with no one [he laughs]. He doesn’t do any harm, to anyone, but the important thing, the important thing is that each time he creates a story for himself [*chaque fois il se crée une histoire*].”

TRANSLATIONS

According to the structure of the clinical interview, Dr. R. moves on to the mother. Dr. R. must collect information about family history and the early signs of the illness. There emerges, at this point, an important issue of translation, predicated on a play of reciprocal projections and on a certain complicity in misrecognition. To proceed, there must be a sense of common ground, a common ground that both parties assume is lacking. The psychiatrist must translate her diagnostic concepts in terms the mother can understand, in vernacular Arabic and in images borrowed from everyday language, without however surrendering to the presumed interpretative schemata of the mother, that of possession by the jinns; the mother must describe her son’s condition in terms the psychiatrist can accept and validate, that is, she must objectify and translate her own experience of his state as if in a foreign language. This double translation produces important alterations that cannot be recognized by

either party. Misrecognized also is the disquieting sentiment that, even when they seem to have reached an agreement on the representation used, saying for instance that “something is ringing in his head,” each interlocutor situates this image in her own frame of reference, which is at odds with the that of the other. They know this, Dr. R. and Reda’s mother, but can’t acknowledge it and perhaps can’t even formulate it. They can’t, because the institution itself, its symbolic efficacy, requires that silence.

And things are even more complex if one considers that the psychiatrist, Dr. R., even though she is external (by choice, by chance, and by historical reason), is not at all foreign to the symbolic universe of Reda’s mother, who could easily be her own mother. This fact raises the unresolved and in a way unspeakable question of cultural belonging as open injury and interrupted transmission. And, at the same time, one can consider that the mother came to the hospital to contain, control, and help her son, but also for a desire impossible to avow, to be able to speak her own rage.

Dr. R. turns then to the mother and asks in Arabic when her son’s illness began. The mother relates how Reda started to change (*keytghiyyer*) two years earlier, when he suddenly embraced religion and took on assiduous praying—even the prayer of dawn, at the mosque, in the middle of winter. What’s wrong with praying? Dr. R. opposes. Attempting to construct her case, the mother speaks of the old men’s surprise at Reda’s sudden religious turn (*key’qjw*), for he used to drink and to smoke. His prayer was not normal (*keysalli sala mashi tabe’ya*). What else was “strange”? asks Dr. R.

“Yes, strange [*ghrib*]!!!” The mother struggles to represent that strangeness: she speaks of her son’s sleepless nights, his aggressive tone, the transformation of his bodily expression. “*Mabqash kima kan* [he was no longer himself],” she concludes.

The psychiatrist provides a vocabulary, “Did he look at himself in the mirror, did he notice a transformation in his image?” The mother says, “Yes, and he would talk to himself, at first he’d stop if we walked in the room, then didn’t care anymore, would just go on speaking to himself in our presence.” And he hit another boy with a knife, she adds. “When?” the psychiatrist asks. “Just a few months ago, and he spent two months in jail, that’s what finished him up.” The psychiatrist interrupts the mother to order, she wants a chronological account, “from beginning to end,” and to ask whether Reda complained about his strangeness, whether he felt the world around him was becoming uncanny, *ghrib*.

“He complained that something was ringing in his head [*katsonne lih shy hajja f-ras*], that he was possessed, ‘there is a jinn in me’ [*keygullik mskun, ‘rah fiya shy jinn’*].”

Dr. R. makes a gesture of recognition, moving toward a possible diagnosis. The mother acknowledges her gesture in silence, with what I read as a sense of

relief. Only later, after the psychiatrist left us alone in the room, she stated that those signs, “that juggling of the limbs and the speaking to himself, the ringing of something in his head,” were signs of demonic possession, and they were not the reason she had come to the hospital. She had come because of Reda’s drug habit, his hashish smoking, because she thought the hashish might have got to his head and, in that case, he would be treated with hospital drugs. In a way, she hoped so. Because in that case doctors could do something; Reda would take medications, and perhaps he’d be cured. But for a demonic possession, the hospital can provide no cure.

Afterward, as I listened to the tape recording of the interview, I asked myself if the truth of the mother was in that discreet revelation of her interpretive universe, which she had omitted in her conversation with Dr. R., or if it was instead in the story she was able to tell after her son’s departure, and because of the active listening of the psychiatrist, in the foreign country of the psychiatric institution.

THE KNOT OF THE SOUL

Later, after her son was escorted to the men’s locked ward, Reda’s mother spoke differently. Her tone changed, and the anguish that froze her as a character, forced to speak the phantasm in the scene of her son’s persecution, loosened into voice and made possible the enunciation of her own injury—the story of her own pain and the way that pain crystallized into the “knot” of her son’s existence. Yet, as stories of suffering in situations of oppression are rarely innocent in their address and rhetorical scope, her story is an ambiguous one, where her pain is both revealed and reinvested in an ultimate appropriation.

It is Dr. R. who can receive her speech within the constraining space of the emergency room. And the mother speaks of a wound, a “knot of the soul,” *l-’aquda nefsyia*, with an evocative expression by which she reckons the intertwining of her suffering with that of her son and laments his fate as an offspring of her own. He is “knotted,” she says, *ma’aqqaḍ*, with an evocative “intermediate” concept, a bridge between two worlds of interpretation—at once an image of bewitchment from the vocabulary of magic and a term for “complex” in that of modern psychology.

The knot of the soul is the hidden wound, physically expressed by the urinary incontinence that accompanied Reda through his adolescence; a wound hidden inside and suddenly exploded to the outside, into the other world of his illness. But the *l-’aquda nefsyia* is also the knot of her own soul: her unhappy marriage, alcoholic husband, the violence of her evenings, the sleeplessness of her nights, the tears. The husband, a policeman who beat her, and Reda, the

little boy who watched the violence, who cried and later interceded for her with his father. The eldest of her children, he saw everything and was traumatized. She uses the technical term *sadama*, “shock” or “trauma,” a term she learned from her children who accused them, the parents, of having traumatized them. Yet despite all this Reda was managing; despite the “knot,” the knot remained hidden and he could succeed in school, the only one of the children to have had an education. But when he got his university degree and didn’t find a job, when he found himself on the street, unemployed, a *chomeur*, all that came out in the open, like a blister, impossible to contain.

Now that the children have grown up, she decided to get a divorce. She was lucky because her husband did not oppose her request. She sought a divorce to try to give a chance to Reda, for the trajectory of his life is cut off. She knows, she says, that it is her own injury that implanted itself into the life of her son and is slowly destroying him. She suggests that I go to visit him in the ward; he will be glad to talk with me.

DON QUIXOTE AND THE CERVANTES STAGE

Sitting on his chair without moving, Reda has been listening in silence to his mother’s description of his state. But when she mentions his experience of possession by a *jinn* he protests, in Arabic, that the *jinns* are also God’s creatures, that they are found everywhere in the world, and that they are mentioned in the Qur’an. The mother opposes that she is just reporting things he said in the past. Reda explodes, in French: “I lied so that she gets away from me, that she leaves me alone, because she’s illiterate, she and her friends are the enemies of science, the enemies of reading! [J’ai menti, pour qu’elle s’éloigne de moi, parce qu’elle est analphabète, elle et ses copines, des ennemies de la science, des ennemies de la lecture].”

The mother talks about his drug habit, his use of hashish, and the recent degeneration of his state. Not sleeping, wandering on the beach, talking about the *jinns*. Reda protests, “C’est le *Stade Cervantes* dans la littérature [it is the Cervantes Stage in literature].”

Irritated, Reda starts speaking a sort of Italian or Spanish (he had asked me where I was from), his own linguistic creation, in which can be recognized the name of Don Quixote. The mother explains that when he’s feeling bad Reda starts speaking other languages, “French, English, Italian, and Berber . . . he’s good with languages. . . .

Dr. R. is moving to a conclusion. The mother hesitates, “I feel I forgot something.” And the psychiatrist asks her whether Reda ever claimed to be a prophet. The mother nods, “Imam l-Mehdi,” she says, “you’re right, I forget some things, once he told me that a woman appeared to him in his sleep and

told him, "You are the son of Myriam, you are l-Imam l-Mehdi, he said this to his father and myself. . . .

Reda protested in French, "This is Cervantes, I am telling you. . . .

The psychiatrist asks him directly, "Are you a prophet?"

"No! That's absurd," he protests, "it is a manner of speaking, like Cervantes [*c'est une façon de dire, comme Cervantes*]. It is manner of living, I do this, every once in a while, it is my way of going on living, time and again, I practice literature in life [*c'est ma façon de vivre, de temps en temps, je fais également de la littérature dans la vie*]. . . .What's wrong with practicing literature in life? I invent, create and recreate [*j'invente et je réinvente*]. . . .

"What is it that you invent?" asks Dr. R., interested, for the first time, in exploring Reda's site of flight, his recurrent recourse from the beginning of the interview.

"Stories such as those she's relating, stories with jinns. They say, for instance, back at home, that certain men have love affairs with she-demons. That's not true, can anyone really describe the procedure of nights [having sex with a jinn]? God created the jinns, and those jinns exist in religion as abstract terms of creature. But no one was ever married to a jinn. It the same thing that happens in literature, when it is applied to the practice of living, every once in a while I use terms such as jinn or jinniya, to live *le Stade Cervantes* in life. Just to get away from those illiterate ones."

"What is this that you call the 'Cervantes Stage'?" asks Dr. R., in Arabic this time, stressing the quotation marks as if not to validate Reda's concept as her own.

"In any case, this is what I call it. It means to make use of imaginary characters at certain times, to speak with the windmills, *tahuna* [mills]," Reda repeats in Arabic, aware that the audience might not be familiar with Don Quixote. "I as well, when I was speaking to myself, I was actually talking to the desk, *imagining* it as an enemy, as Don Quixote did. . . .

Dr. R. stops listening, asks the mother whether they ever took Reda to the hospital or to a *fqih*. At his mother's reply he becomes angry. Siding with Reda for the first time, Dr. R. addresses the mother in a reproachful tone. Dr. R. is moved in her modernist sensibility closer to Reda, in fact, in her own experience: who are the healers her son is complaining about?

"They are well known in town," says the mother defensively. I ask her what was the healer's diagnosis: *mqius*, she says, "touched" (by the jinns), and starts talking about his excessive washing as evidence of possession (demons are said to like water). Reda opposes that it is his right to wash, to be clean, it is called hygiene.

A nurse comes in, and escorts Reda to the locked ward. Dr. R's quick notes, which will never become a formal patient file, mention psychotic symptoms, pointing to a psychotic personality and possibly an early onset of schizophrenia.

Later, at the ward, Reda is brought down by a nurse, who waits outside the door because the patient is “not cooperating” and opposes his hospitalization. Alone for the first time, we talk in a room by the main patio—about literature, Don Quixote, and what makes it difficult for him to concentrate. He asks me about my nationality, he already knew in fact, and starts speaking a sort of invented Italian that turns into Spanish and finally breaks into incomprehensible speech. Language hardens as a wall between us, and against this wall our conversation dissolves. When I come back to visit him the next morning, I learn that he has fled the hospital, climbing a high wall.

THE INTERMEDIATE WORLD

Aware of the risks of speculation (reducing Reda’s suffering to my desire to understand), of that particular sort of appropriation that is anthropological interpretation, I attempt to reflect on the status of that other place, the Stage, where Reda retreats (*kanharrah*) and from which he fights back (*kanharrah*), in the rough moments of his illness as during the ER interview; the Stage to which he bestows the status of a theory, in which he is both a character and the director, and which is the site—rhetorical, existential, and physical—of his surrender to the Other and his desire for self.

I am encouraged in this reflection by the ambiguity of my position, a position I so often experienced as frustrating and even paralyzing, always external to the action, if also enmeshed in it (the anthropologist listening, asking questions, feeling directly concerned, affected, yet unable and unauthorized to “cure”), but which can be enabling of an oblique interpretative space, a space of “proximity,” marginal to action and decision, a peripheral zone of speculation and critique. In attempting to inhabit this space, I am inspired by the strategy inaugurated by Reda himself, his “appropriation” of Don Quixote, which is also a dispossession of himself, and his multilayered use of characters and rhetorical figures by which, like the Spanish *hildago*, he labors at creating a multidimensional space where his uncanny experience of the world may find expression and elaboration. In this sense, his strategy can be called, as he calls it, literary.

Literature, writes Deleuze in one of his last essays, is the opposite of phantasms. Phantasms fix identities in place, personalize characters and seal possession, while literature is a process of dispossession, a becoming-other that dwells in a zone of proximity (*voisinage*), and is never saturated by an identity: “literature begins only when a third person is born in us that strips us of the power to say I” (Deleuze 1998: 3). In this sense, Deleuze says, literature is health—a vulnerable health (*une petite santé*) that almost choked in its experi-

ences, but that gained in that process the capacity of becoming. It is born from within delirium, as a movement of straying, becoming foreign, from the mother tongue (from the Mother), from the madness of ideological identifications, and it transforms them from within, such an antidelirium; a becoming other that opens onto “visions,” “ideas” that are not the projection of an Ego and that can become the singular-collective utterance of a people to come.

Reda has discovered his literary “procedure” in the work of Cervantes, where he encountered his own story in an exemplary form, translated into a foreign language. Thanks to this discovery, which coincided with his first realization of a feeling of strangeness in his experience of the world, he has taken on the practice of literature, which he intends from the point of view of the character as well as of that of the author. It is not by chance that in his literary search through Voltaire and Dostoevsky in the town library of Tiflet Reda settled on Don Quixote, the text that raises the ambiguity of character and author, ironical distancing and imaginary capture, life and fiction, to the principle of its narrative structure.

(Reaching back stenographically to the political questions raised earlier on in the discussion, it could be said that “literature,” in Reda’s use and as read through Deleuze, discloses an unforeseen possibility of *twkkel*, the murderous “us” of the collective identification. Literature opens for Reda the possibility of negotiating his debt to a failing cultural transmission in the form of a not-yet or no-longer us. While the objectifications of his modernity ideal are rigid, as the saturation of knowledge in a self-sufficient totality, the moving and multiple identifications of the *Stade Cervantes* are not.)

To phrase the question in terms closer to the clinical concerns of psychiatrists, it would be wrong to reduce Reda’s procedure to the deployment of a delusion or to the commonly observed quest of patients at the onset of psychosis, trying to make sense and “rationalize” their experiences of growing strangeness in terms of a fantasy, which is then systematized into a delusion. That reduction would miss the dimension of struggle in Reda’s experience, obliterating the sense in which his search is meaningful and his experience of otherness less unfamiliar than one may be willing to acknowledge. This was also the reaction of the psychiatrists at the hospital when we discussed a draft of this chapter: expressing the question in a diagnostic idiom, they said that if Reda had most likely a psychotic personality, he was however not fully psychotic, thanks to an element of creativity and to his ability to self-reflect.

First his symbolic geography. The Stage, as a topological elsewhere, is an effect of withdrawal, of breaking the ties with “home” (*chez soi*) in all its forms: the family, the home town, vernacular Arabic (the mother tongue), the space of the everyday, of social relations and tradition. It is the result of a

radical disaffiliation of which Reda is at once the victim, the character, and the author. This “retreat” is a vital distancing from the mother, figure of the uninhabitable home, and from the magical logic of *twkkel*, the envious incorporation of the other. But the distancing is more than just a withdrawal. It is a departure, a journey, the metamorphosis of the self in a theater of imaginary forms, where the possibility of self-realization gambles with the risk of nonreturn.

On the one hand, *le Stade Cervantes* is for Reda the ironic moment of a coming of age, when the break with the forms of a fossilized tradition is consummated and “creation” becomes possible: “*j’invente and je réinvente*.” The Cervantes Stage is the stage of “maturity” Reda reached in his evolution, inaccessible for his illiterate parents and from which are excluded all those who seek the reassurance of tradition. On the other hand, the “Stage,” as Reda describes it, is also a scene of alienation, the fact of becoming a character in one’s creation: “*Je fais de temps en temps de la littérature dans la vie* [time and again, I practice literature in life].” Reda becomes Don Quixote who, like himself, has read “all the books of history” and has given up all ties for a life of travel in the meadows of the imagination; Don Quixote who, like himself, lives at a time lag with his contemporaries, a temporal disjunction, speaks a “dead language” full of references they don’t understand, because he himself has renounced their recognition. But Don Quixote is tricked by his imagination. Reda says this in full knowledge, as he looks at himself from the critical perspective of the author. He smiles and says that it happens to him too, sometimes, to fight against the windmills: he calls it living the Cervantes Stage in real life. Suspended between the author and the character, Reda doesn’t seek a resolution: invention is something else than mastery, it is to practice literature in life, “to create stories for oneself.” He claims for himself his *j’invente* as an exercise of self-fashioning, struggling to find balance on the line between delusion and creation.

Yet in his conception of the Stage there is the trace of a possible mediation. To the psychiatrist’s question, “What do you invent?,” Reda replies that he invents stories of jinns, jinns that he enacts in the *mise-en-scène* of voices to live the Cervantes Stage in real life; jinns that he glosses as “abstract terms of creatures” (in the Qur’an) and compares to figures of speech, styles of speaking, “*manières de parler*,” when one practices literature in life. At once a space of language, where words materialize as images, and a mystical space, populated with jinns, *le Stade Cervantes* is an intermediate world, realm of the creative imagination: in Ibn al-ʿArabi’s Islamic metaphysics, *Al-Alam al-Mithal*. It becomes then the site of a possible mediation, a passageway between delusion and creation, where the rejected terms of cultural identification, the remains of an inaccessible tradition, are reencountered and can be symbolically transformed in that “foreign” space. For a moment the threatening otherness of the persecutory interpretation (the

poisonous communal meal), becomes for Reda an ecstatic experience: “*inquiétante familiarité, une fréquentation de “l’Autre”*” (Certeau 1982: 20).

NOTES

I would like to thank the staff of the Hospital Arrazi in Salé, Morocco, and particularly Dr. Mehdi Paes, Jamaladdine Ktiouet, Selwa Kjiri, Jalal Toufiq, and Mustapha Laymani. Thanks also to Fouad Bencheikroun, Abdellah Hammoudi, Mohammed Hamdouni Alami, Luca D’Isanto, Veena Das, M. Letizia Cravetto, Saba Mahmood, Lawrence Cohen, Eric Glassgold, Andrew Lakoff, Baber Johansen, M. Pia Di Bella, Byron Good, Mary-Jo DelVecchio Good, and Pete Skafish. Research on psychiatry in Morocco in 1998–99 was funded by a grant of the Social Science Research Council and in 2001–02 by a grant from the Sultan Committee, CMES, UC Berkeley.

1. See in this context Pandolfo (2006a, 2006b); on the history and predicament of colonial psychiatry, see Pandolfo (2000).
2. Article 11 reads, “L’hospitalisation dans un service public ou un établissement privé de psychiatrie ne peut avoir lieu qu’au vu d’un certificat délivré par un médecin psychiatre qualifié mentionnant de façon détaillée et précise les anomalies du comportement du malade et concluant à la nécessité de l’hospitalisation [Hospitalization in a public or a private psychiatric institution can exclusively take place at the production of a certificate issued by a qualified medical psychiatrist, describing in a detailed and precise manner the anomalies of the patient’s behavior, and concluding to the need of hospitalization] (my translation).
3. The question remains open of how to understand this process from the point of view of psychic life, or what some psychoanalysts have called the institution and deinstitution of the subject. In a related context, psychoanalysts writing on exile and migration stress the crucial clinical dimension of “relating to the break,” the trauma, as a recognition of radical discontinuity that is prior to the possibility of reinhabiting a space (cf. Deleuze 2001).
4. One of the prime consequences of the World Bank program of structural adjustment for Morocco in the 1980s has been the cutting of all forms of aid or subsidy for health care.
5. The French word *chomeur* means “unemployed” or “jobless person.” In Morocco this is today a charged term, at once scornful and expressing a social and political discontent. Since 1990 there has existed a countrywide organized movement of “Chômeurs Diplômés,” which has become symbolic of a generalized sense of distress and discontent among the youth. The senior psychiatrists commented at the staff meeting, “This patient’s case helps us clarify the theme of *chomage*.”
6. Morocco has since had the experience of a truth commission, which concluded its proceedings in February 2006. L’Instance Équité et Reconciliation (IER; *Al-Insaf wa al-musalaha*) was created in April 2004 by royal decree, with the mandate to investigate and record human rights violations committed since national independence

and during the kingdom of the late king Hassan II (1956–99). The IER collected hundreds of testimonies of “victims” (*al-dahia* is the term officially used) or relatives of deceased victims and received over twenty thousand victims’ requests of reparation. In spring 2005 public hearings were held by the IER in several cities, covered by the Moroccan and international media. The process stirred many hopes and a heated debate, with complex and unintended outcomes. Yet in the Moroccan situation the equation of witnessing, injury, and citizenship (based on human rights) is less obvious than in other international contexts. Among large unprivileged sectors of the Moroccan population, less conversant than the political elite with the international idiom of human rights, skepticism colored people’s perception of the work of the IER, as well as the notion of accessing citizenship through witnessing.

7. I am referring here to the clinical-theoretical work on the *espace originaire*, or “primal space” (Aulagnier 1975), and the archaic experience of the body in psychosis and schizophrenia (Pancow 1969), as well as of recent contributions in cultural psychiatry (Jenkins and Barrett 2004, particularly J. Jenkins, chapter 1, “Schizophrenia as a Paradigm Case for Understanding Fundamental Human Processes,” and E. Corin, chapter 4, “Living through a Staggering World: The Play of Signifiers in Early Psychosis in South India”). See also Corin (1990).
8. All names of persons in this essay, and specifically those of patients and psychiatrists, have been changed—unless reference is made to psychoanalysts or psychiatrists as authors of published texts.
9. The Moroccan university curriculum for the scientific disciplines, including mathematics (Faculté des Sciences), is entirely in French. The social sciences, as well as literature and philosophy, are instead taught in Arabic. At the Schools of Medicine, Architecture, and Engineering, instruction is in French.
10. The struggle for place, for horizon, for distance, is a central feature in the experience of youths with whom I spoke in poor neighborhoods. As I write about Reda, I think of a youth recounting to me his nights, sitting on top of a hill overlooking the freeway, in the silence in which one can breathe, following the movement of long-distance trucks with his binoculars, waiting for the truck in which he will hide, to risk his way beyond the Spanish border. See Pandolfo (2007).
11. In an essay on Schreber’s *Memoir of My Nervous Illness*, Vincent Crapanzano (1988) describes a comparable struggle against annihilation, in which “space,” “occupation,” and “distance” play a fundamental role.
12. Abdelfattah Kilito, personal communication, February 2002.
13. See chapter 2, “The Historiographical Operation,” in Certeau (1988).

REFERENCES

- Agamben, G. 1998. *Homo Sacer: Sovereign Power and Bare Life*. Trans. D. Heller-Roazen. Stanford, CA: Stanford University Press. Originally published as *Homo Sacer: Il potere sovrano e la nuda vita* (Torino: Einaudi, 1995).

- Arendt, H. 1971. *The Origins of Totalitarianism: Imperialism*. New York and London: Harcourt.
- Asad, T. 2002. Interview by Nermeen Shaikh. *Asia Source*, December 16. www.asiasource.com/news/special_reports/asad.cfm.
- Aulagnier, P. 1975. *La violence de l'interprétation*. Paris: PUF.
- Benslama, F. 1988. *La nuit brisée: Muhammed et l'énunciation Islamique*. Paris: Ramsay.
- . 2000. épreuves de l'étranger. In "Clinique de l'exil," *Cahier Intersignes* 14/15: 9–29.
- . 2002. *La psychanalyse à l'épreuve de l'Islam*. Paris: Aubier.
- Berthelie, R. 1994. *L'homme maghrebin dans la littérature psychiatrique*. Paris: l'Harmattan.
- Binswanger, L. 1958. The Case of Ellen West. In *Existence: A New Dimension in Psychiatry and Psychology*, ed. R. May, E. Angel, H. Ellenberg, 237–64. New York: Basic Books.
- Borch-Jacobsen, M. 1993. *The Emotional Tie*. Trans. D. Brick. Stanford, CA: Stanford University Press. Originally published as *Le lien affectif* (Paris: Aubier, 1991).
- Certeau, M. de 1982. *La fable mystique*. Paris: Gallimard.
- . 1988. *The Writing of History*. Trans. Tom Conley. New York: Columbia University Press. Originally published as *L'écriture de l'histoire*, (Paris: Gallimard, 1975).
- Corin, E. 1990. Facts and Meaning in Psychiatry: An Anthropological Approach to the Lifeworld of Schizophrenics. In *Culture, Medicine and Psychiatry* 14: 153–88.
- Deleuze, G. 1998. *Critical and Clinical Essays*. Trans. D. Smith and M. Greco. Minneapolis: University of Minnesota Press. Originally published as *Critique et Clinique* (Paris: éditions de Minuit, 1993).
- Crapanzano, V. 1998. Lacking Now is Only the Leading Idea, That Is—We, the Rays, Have No Thought: Interlocutory Collapse in Daniel Paul Schreber's *Memoirs of my Nervous Illness*. *Critical Inquiry* 24 (Spring): 737–67.
- Fassin, D. 1999. Santé et immigration: Les vérités politiques du corps. *Cahiers de URMIS* 5: 69–76.
- . 2000. Politique du vivant et politique de la vie: Pour une anthropologie de la santé. *Anthropologie et Société*, 24 (1): 95–116.
- . 2001. The Biopolitics of Otherness: Undocumented Foreigners and Racial Discrimination in French Public Debate. *Anthropology Today* 17 (1): 3–7.
- Favret-Saada, J. 1977. *Les mots, le mort, les sorts: La sorcellerie dans le bocage*. Paris: Gallimard.
- Igert, M. 1955. Introduction à la psychopathologie marocaine. *Maroc Médical*, no. 360: 1309–32.
- Jenkins, J., and R. Barrett, eds. 2004. *Schizophrenia, Culture, and Subjectivity*. Cambridge: Cambridge University Press.

- Lacoue-Labarthe, P., and J.-L. Nancy. 1997. La panique politique. In *Retreating the Political*, 1–31. New York: Routledge.
- Pancow, G. 1969. *L'homme et sa psychose*. Paris: Flammarion.
- Pandolfo, S. 2000. The Thin Line of Modernity: Reflections on Some Moroccan Debates on Subjectivity. In *Questions of Modernity*, ed. T. Mitchell, 115–47. Minneapolis: University of Minnesota Press.
- . 2006a. Bghit nghanni hnaya (Je veux chanter ici): Voix et témoignage en marge d'une rencontre psychiatrique. *Arabica* 53 (2): 232–80.
- . 2006b. Nibtidi minin il-hikaya (Where Are We to Start the Tale): Violence, Intimacy, and Recollection. *Social Science Information* 45 (3): 349–71.
- . 2007. The Burning: The Theologico-Political Imagination of Illegal Migration. In "Transitional Transcendence," special issue, *Anthropological Theory* 7 (3).
- Prakash, G. 1999. *Another Reason: Science and the Imagination of Modern India*. Princeton, NJ: Princeton University Press.
- Roland, J. L. 1957. De quelques incidences psycho-pathologiques des phénomènes de surpopulation: Vue d'ensemble; Aspect propre du Maroc. *Maroc Médical*, no. 387 (Août).